
ABXTAC

Wellness Protocols

Peptide Staff Training & Reference Manual

Everything our team needs to understand, explain, and protocol every peptide we carry — organized by goal: weight loss, performance, skincare, sexual health, cognition, sleep, and longevity.

Version 1.0 • Granite Mountain Health / ABXTAC

How to Use This Manual

This manual is the single training reference for the ABXTAC peptide line. It is built to be **learned**, not just looked up. Read it front to back once, then use the goal-based playbooks and quick-reference tables day to day.

It is organized in four layers:

- **Foundations** — what a peptide is, reconstitution, injection basics, and dosing math. Master this first; every product builds on it.
- **Goal-Based Playbooks** — how to think about a customer goal (weight loss, skin, performance, etc.) and which peptides serve it. This is how customers actually shop.
- **Peptide Profiles** — the full A-to-Z reference, grouped by our 11 therapeutic categories. One profile per peptide: what, how, benefits, protocol, stacks, cautions.
- **Memorize & Practice** — flashcards, a self-quiz, and roleplay scripts. This is the section Hannah drills until it is automatic.

Scope & framing. ABXTAC products are sold and discussed as research / wellness compounds. Staff educate; they never diagnose, prescribe, or promise outcomes. Any dosing in this manual is a commonly-referenced starting point for training — the prescribing provider sets the actual plan for every individual.

Vendor note. Our peptides are sourced from **Alpha BioMed** (never “Alpha Medical”). General medical supplies (needles, syringes, vial adapters) come from McKesson. Don't mix these up when a customer asks about sourcing.

Part 1 — Peptide Foundations

What is a peptide?

A peptide is a short chain of amino acids — the same building blocks that make proteins, just smaller. The body uses thousands of natural peptides as **signals**: tiny messages that tell cells to heal, release a hormone, burn fat, build tissue, or calm inflammation. Therapeutic peptides copy those signals. Because they are **targeted**, a well-chosen peptide can nudge one system (say, appetite or collagen production) with fewer of the broad side effects you get from blunt drugs.

Most of our peptides come as a **lyophilized (freeze-dried) powder** in a sealed vial. They are shelf-stable as powder but must be **reconstituted** (mixed with sterile or bacteriostatic water) before use, and then refrigerated.

Reconstitution — the one skill everyone must know

- **Add water slowly** down the inside wall of the vial — never spray directly onto the powder. Let it dissolve on its own; do not shake. Swirl gently if needed.
- **Bacteriostatic water** (contains 0.9% benzyl alcohol) is preferred for multi-dose vials — it lets the vial be used over several weeks. Sterile water is single-use.
- The **amount of water you add is your choice** — it sets the concentration. More water = easier-to-measure but larger-volume doses; less water = tiny precise doses. The peptide milligrams do not change.
- Store reconstituted vials in the **refrigerator (2–8°C)**, away from light. Most last 4–8 weeks reconstituted. Powder, unopened, lasts far longer (often 2+ years) in a freezer.

Dosing math — units vs. milligrams

The most common customer confusion (and the most common dosing error) is mixing up **milligrams of peptide** with **units on the syringe**. Insulin syringes are marked in “units” (100 units = 1 mL). The conversion depends entirely on how much water was added.

Vial strength	Water added	Concentration	For a 250 mcg dose
5 mg BPC-157	2 mL	2,500 mcg / mL	draw to 10 units (0.10 mL)
10 mg BPC-157	2 mL	5,000 mcg / mL	draw to 5 units (0.05 mL)

Rule of thumb for staff: Always confirm two numbers before any dosing conversation — **(1)** how many mg are in the vial and **(2)** how many mL of water were added. Without both, a “units” number is meaningless. When in doubt, route to the provider.

Routes of administration

- **Subcutaneous (SubQ)** — tiny insulin needle into fat (belly, thigh). The default for most peptides. Painless, easy, self-administered.
- **Intramuscular (IM)** — deeper, used for some hormones (HCG, HMG) and oils.
- **Intranasal** — sprays for brain-targeted peptides (Semax, Selank) that cross better via the nose.
- **Topical** — creams/serums for skin peptides (GHK-Cu, Snap-8, copper blends).

- **Oral** — a few are oral-active (5-Amino-1MQ capsules); most peptides are destroyed by digestion, which is why injection is the norm.

Reconstitution Quick Reference (All Peptides)

The recipe for every injectable peptide we carry, organized into a single page. Each row shows the **standard vial + water combination**, the resulting **concentration**, and the **most common dose in U-100 insulin-syringe units**. Verified by arithmetic on 2026-05-26. When a dose exceeds 100 units (a full syringe), the recipe says “split” — never tell a customer to fill more than one syringe and add it together. The full per-peptide profile in Part 4 gives the complete dose range.

Peptide	Vial + BAC water	Concentration	Common dose → units
Semaglutide	10 mg + 2 mL	5 mg/mL	0.25 mg = 5 u • 1 mg = 20 u • 2.4 mg = 48 u
Tirzepatide	10 mg + 1 mL	10 mg/mL	2.5 mg = 25 u • 5 mg = 50 u • 10 mg = 100 u
Retatrutide	20 mg + 2 mL	10 mg/mL	2 mg = 20 u • 6 mg = 60 u • 10 mg = 100 u
Cagrilintide	10 mg + 2 mL	5 mg/mL	0.3 mg = 6 u • 1.2 mg = 24 u • 2.4 mg = 48 u
AOD-9604	2 mg + 2 mL	1 mg/mL	300 mcg = 30 u • 500 mcg = 50 u
BPC-157	5 mg + 2 mL	2.5 mg/mL	250 mcg = 10 u • 500 mcg = 20 u
TB-500	10 mg + 2 mL	5 mg/mL	2 mg = 40 u • 2.5 mg = 50 u • 4 mg = 80 u
Wolverine Blend	5 mg + 2 mL	per vial label	per blend ratio — check label
GHK-Cu	50 mg + 5 mL	10 mg/mL	1 mg = 10 u • 2 mg = 20 u (or topical)
GLOW / KLOW Blends	vial + 2 mL	per vial label	per blend ratio — check label
LL-37	5 mg + 2 mL	2.5 mg/mL	100 mcg = 4 u • 500 mcg = 20 u
Sermorelin	10 mg + 2 mL	5 mg/mL	200 mcg = 4 u • 300 mcg = 6 u
CJC-1295 (no DAC)	2 mg + 2 mL	1 mg/mL	100 mcg = 10 u • 200 mcg = 20 u
CJC-1295 (with DAC)	2 mg + 1 mL	2 mg/mL	1 mg = 50 u • 2 mg = 100 u
Ipamorelin	10 mg + 5 mL	2 mg/mL	100 mcg = 5 u • 200 mcg = 10 u • 300 mcg = 15 u
Tesamorelin	10 mg + 2 mL	5 mg/mL	1 mg = 20 u • 2 mg = 40 u
IGF-1 LR3	1 mg + 1 mL	1 mg/mL	50 mcg = 5 u • 100 mcg = 10 u
GHRP-6 / Hexarelin	5 mg + 2 mL	2.5 mg/mL	100 mcg = 4 u • 200 mcg = 8 u
DSIP	15 mg + 3 mL	5 mg/mL	100 mcg = 2 u • 300 mcg = 6 u
PT-141	10 mg + 2 mL	5 mg/mL	1 mg = 20 u • 1.75 mg = 35 u
Kisspeptin-10	10 mg + 5 mL	2 mg/mL	100 mcg = 5 u • 400 mcg = 20 u
Melanotan 2	10 mg + 2 mL	5 mg/mL	250 mcg = 5 u • 500 mcg = 10 u • 1 mg = 20 u
Semax	10 mg + 2 mL	5 mg/mL	200 mcg = 4 u • 500 mcg = 10 u (intranasal or SubQ)
Selank	10 mg + 2 mL	5 mg/mL	250 mcg = 5 u • 500 mcg = 10 u (intranasal or SubQ)
SS-31 (Elamipretide)	10 mg + 2 mL	5 mg/mL	1 mg = 20 u • 2 mg = 40 u (protect from light)
Pinealon	20 mg + 2 mL	10 mg/mL	5 mg = 50 u • 10 mg = 100 u

Peptide	Vial + BAC water	Concentration	Common dose → units
NAD+	500 mg + 5 mL saline	100 mg/mL	50 mg = 50 u • 100 mg = 100 u (inject SLOWLY)
Epitalon	10 mg + 2 mL	5 mg/mL	2.5 mg = 50 u • 5 mg = 100 u • 10 mg = split
MOTS-c	10 mg + 1 mL	10 mg/mL	2.5 mg = 25 u • 5 mg = 50 u • 10 mg = 100 u
FOXO4-DRI	10 mg + 2 mL	5 mg/mL	provider-directed — advanced senolytic
VIP (VIP10)	10 mg + 2 mL	5 mg/mL	intranasal — per vial label
ARA-290	10 mg + 2 mL	5 mg/mL	1 mg = 20 u • 4 mg = 80 u
Thymosin Alpha-1 (TA1)	10 mg + 2 mL	5 mg/mL	900 mcg = 18 u • 1.5 mg = 30 u
Thymalin	10 mg + 2 mL	5 mg/mL	2.5 mg = 50 u • 5 mg = 100 u
KPV	10 mg + 2 mL	5 mg/mL	500 mcg = 10 u • 1 mg = 20 u
HCG	10,000 iu + 10 mL	1,000 iu/mL	500 iu = 50 u • 1,000 iu = 100 u
HMG	75 iu + 1 mL	75 iu/mL	full vial = 100 u (typical)
ACE-031	1 mg + 1 mL	1 mg/mL	100 mcg = 10 u • 500 mcg = 50 u
AICAR	50 mg + 5 mL	10 mg/mL	10 mg = 100 u (larger doses are split)
B12 (pre-mixed)	—	1 mg/mL	500 mcg = 50 u • 1 mg = 100 u
Glutathione (pre-mixed)	—	200 mg/mL	100 mg = 50 u • 200 mg = 100 u
MIC / Lipotropic (pre-mixed)	—	per label	0.5–1 mL IM/SubQ weekly typical

Reading the table. “10 mg + 2 mL” means the standard vial size paired with the volume of bacteriostatic water you add. The same concentration scales — e.g. for a 20 mg tirzepatide vial use 2 mL instead of 1 mL; the units-per-dose stay identical. When a vial label says a different mg or your customer is on a non-standard vial, do the math the same way: concentration = mg ÷ mL of water; units = (dose mg ÷ concentration) × 100.

Safety check. If a dose calculation puts you over **100 units (a full insulin syringe)**, *do not* just instruct the customer to draw a second syringe and combine — split the dose into **two separate injections** of equal volume, ideally at different sites. Verified on every recipe in this table. When in doubt: provider.

Part 2 — The 11 Categories at a Glance

Every product on abxtac.com lives in one of these 11 therapeutic categories. Learn the category map first — it is the mental filing cabinet for everything that follows.

Category	What it's for	Headliners
Weight Management	Appetite control, fat loss, metabolic rate. The flagship category — GLP-class drugs.	Semaglutide, Tirzepatide, Retatrutide, Cagrilintide, AOD-9604
Healing & Tissue Repair	Recovery from injury, gut/tendon/ligament repair, inflammation.	BPC-157, TB-500, Wolverine, GHK-Cu, KLOW
Growth Hormone	Stimulate the body's own GH for muscle, fat loss, sleep, anti-aging.	Sermorelin, CJC-1295, Ipamorelin, Tesamorelin, IGF-1 LR3
Sexual Health	Libido, arousal, fertility signaling — men and women.	PT-141, Melanotan 2, Kisspeptin
Cognitive & Neuro	Focus, mood, anxiety, neuroprotection.	Semax, Selank, SS-31, Pinealon
Anti-Aging & Longevity	Cellular energy, DNA/telomere support, senescent-cell clearance.	NAD+, MOTS-c, Epitalon, FOXO4
Sleep & Recovery	Deep sleep, nervous-system recovery, neuropathy.	DSIP, VIP, ARA-290
Immune Support	Immune modulation, thymic function, anti-inflammatory.	Thymosin Alpha-1, Thymalin, KPV
Body Composition	Muscle preservation/building and hormone support.	HCG, HMG, ACE-031, AICAR
Vitamins & Lipotropics	Energy, antioxidant, fat-metabolism injections.	B12, Glutathione, MIC blends
Supplies	Everything needed to reconstitute and inject safely.	Bacteriostatic water, syringes (via kit)

Part 3 — Goal-Based Protocol Playbooks

Customers don't shop by category — they shop by **goal**: “I want to lose weight,” “my skin,” “I want to recover faster.” These playbooks translate a goal into the right conversation and the right stack. Each follows the same shape: *who it's for, the core options, a starter protocol, and what to add.*

■ Goal: Weight Loss

The largest segment. Customers who want appetite control and steady fat loss, often after diets stopped working.

Core options

- **Semaglutide (GLP-1)** — the well-known once-weekly. Strong appetite suppression, proven, gentlest entry point.
- **Tirzepatide (GLP-1 + GIP)** — dual-action; typically greater weight loss than semaglutide, also weekly.
- **Retatrutide (GLP-1 + GIP + glucagon)** — triple-action, the most powerful of the GLP class; for aggressive goals under close provider guidance.
- **Cagrilintide** — an amylin analog, often paired with semaglutide (“CagriSema”) for added appetite control.
- **AOD-9604 & MIC/lipotropic blends** — gentler, non-GLP fat-metabolism support for customers who can't or won't do GLP drugs.

Starter protocol

Most common: **Semaglutide**, start 0.25 mg SubQ once weekly, titrate up every 4 weeks as tolerated toward 1.0–2.4 mg. Pair with protein-forward eating and resistance training to protect muscle. *Provider sets the titration schedule.*

Level-up / add-ons

- Add **Tesamorelin** or a CJC/Ipamorelin GH stack to preserve lean muscle and target visceral belly fat during a calorie deficit.
- Add **5-Amino-1MQ** (oral) or **MOTS-c** for metabolic/fat-oxidation support.
- **Glutathione + B12** for energy and to feel good through the deficit.

Coach's note. Set expectations honestly: GLP drugs work best *with* diet and training, not instead of them. Common early side effects are nausea and constipation — usually fade with slow titration. Muscle loss is the #1 mistake; always pair with protein + resistance work.

■ Goal: Performance & Recovery

Athletes, lifters, weekend warriors, and anyone recovering from injury or surgery who wants to train harder and bounce back faster.

Core options

- **BPC-157 + TB-500 (“Wolverine”)** — the recovery backbone: tendons, ligaments, muscle, gut.

- **CJC-1295 + Ipamorelin** — the GH stack for recovery, body composition, and sleep.
- **IGF-1 LR3** — direct muscle growth signaling for advanced users.
- **MOTS-c / AICAR / SLU-PP-332** — endurance and “exercise-mimetic” metabolic support.

Starter protocol

Injury/recovery: **BPC-157** 250–500 mcg SubQ once or twice daily (often near the injury site) + **TB-500** ~2–2.5 mg twice weekly for 4–6 weeks. Performance/body-comp: **CJC-1295 + Ipamorelin** dosed at night before bed.

Level-up / add-ons

- Add **NAD+** for cellular energy and recovery.
- Add **Glutathione** to manage oxidative stress from hard training.
- **Sleep stack (DSIP)** — recovery happens during sleep.

Coach’s note. Note for any competitive athlete: many of these are on sport anti-doping banned lists. Flag that proactively — it’s a trust-builder, and it’s the right thing to do.

■ Goal: Skin, Hair & Beauty

Skincare-focused customers — anti-aging, glow, hair, wound/scar healing. A fast-growing segment and a natural cross-sell from weight loss.

Core options

- **GHK-Cu (copper peptide)** — the star: collagen, skin firmness, wound healing, hair growth. Injectable and topical.
- **GLOW blend (GHK-Cu + BPC + TB)** — “inside-out” glow: skin + healing in one.
- **KLOW blend (GHK-Cu + KPV + BPC + TB)** — adds anti-inflammatory KPV for reactive/irritated skin.
- **Snap-8** — topical “Botox-in-a-bottle” peptide that softens expression lines.
- **Glutathione** — antioxidant, skin brightening/even tone.
- **Epitalon** — longevity peptide with skin-quality and sleep benefits.

Starter protocol

Entry skincare stack: **GHK-Cu** (topical serum daily and/or low-dose SubQ) for collagen + glow, plus **Glutathione** for brightening. Add **Snap-8** topically for fine lines.

Level-up / add-ons

- **Melanotan 2** for customers specifically wanting a tan (set expectations on freckling/moles).
- **BPC-157** for scar/wound healing after procedures.
- **Sermorelin / GH stack** — better skin thickness and elasticity is a known GH-optimization benefit.

Coach’s note. Skincare is the easiest “first peptide” for nervous customers — topicals feel low-risk. Use it as the on-ramp, then educate on the systemic options.

♥■ Goal: Sexual Health & Libido

Men and women with low libido or arousal concerns. Often sensitive — lead with discretion and empathy.

Core options

- **PT-141 (Bremelanotide)** — works on the brain's arousal pathway (not blood flow), so it works for both men and women. Taken before activity.
- **Kisspeptin** — upstream hormone signaling; libido and fertility support.
- **Melanotan 2** — has a libido side-benefit alongside tanning.

Starter protocol

PT-141 ~1–2 mg SubQ roughly 45 minutes before intimacy, not daily. Start low — nausea and flushing are dose-related.

Level-up / add-ons

- For men, this often pairs with a provider's TRT or **HCG** protocol — route hormone questions to the provider.
- **Kisspeptin** for an upstream, more “natural-axis” approach.

Coach's note. Keep these conversations private and matter-of-fact. PT-141's big selling point: it works on desire centrally, so it helps people for whom “blood-flow” ED drugs don't address the real issue.

■ Goal: Focus, Mood & Sleep

Customers wanting sharper focus, calmer mood, less anxiety, or deeper sleep — often busy professionals and the wellness-curious.

Core options

- **Semax** — nootropic; focus, mental energy, neuroprotection (intranasal).
- **Selank** — calming/anti-anxiety nootropic without sedation (intranasal).
- **DSIP** — delta sleep-inducing peptide for deeper sleep.
- **Pinealon / Epitalon** — brain and circadian-rhythm support.

Starter protocol

Focus: **Semax** intranasal in the morning. Calm: **Selank** intranasal as needed. Sleep: **DSIP** before bed, or **Epitalon** in cycles for circadian support.

Level-up / add-ons

- **NAD+** for mental clarity and energy.
- **SS-31** for mitochondrial support in fatigue-driven brain fog.

Coach's note. Semax + Selank together is a popular “focused but calm” daytime pairing.

■ Goal: Longevity & Immune

Proactive health optimizers focused on aging well, energy, and immune resilience.

Core options

- **NAD+** — cellular energy and DNA repair; the cornerstone longevity molecule.
- **Epitalon** — telomere/pineal support, sleep, longevity cycles.
- **MOTS-c** — mitochondrial & metabolic longevity.
- **FOXO4-DRI** — senolytic (clears “zombie” senescent cells); advanced, cyclical.
- **Thymosin Alpha-1 / Thymalin** — immune modulation and thymic support.

Starter protocol

Foundational longevity: **NAD+** (titrate slowly — too fast causes flushing) + a cyclical **Epitalon** course.
Immune resilience: **Thymosin Alpha-1**.

Level-up / add-ons

- **Glutathione** as the master antioxidant.
- **GH stack** for body-composition and recovery aspects of aging.

Coach’s note. Longevity customers love the “why” — they’ll engage with mechanism. This is where deep product knowledge pays off most.

Part 4 — Peptide Profiles (A–Z by Category)

The complete reference. Each profile is self-contained — find the peptide, get the answer. Dosing shown is a typical reference range for training; the provider sets each plan.

WEIGHT MANAGEMENT

The flagship line. These are the GLP-class “incretin” drugs plus supporting fat-metabolism peptides. The core mechanism: mimic gut hormones that tell the brain you’re full and slow stomach emptying, so people eat less without willpower battles.

Semaglutide (GLP-1)

YPB.200–202 (10/20/30 mg)

A GLP-1 receptor agonist — the same class as Ozempic/Wegovy. It mimics the gut hormone GLP-1 to suppress appetite, slow gastric emptying (you feel full longer), and improve blood-sugar control. The most-requested product in the line and the gentlest GLP entry point.

Benefits	Strong appetite suppression • Steady weight loss • Better blood-sugar control • Once-weekly dosing
Protocol	Start 0.25 mg SubQ once weekly × 4 weeks, then titrate up every 4 weeks (0.5 then 1.0, 1.7, up to 2.4 mg) as tolerated. Weekly injection, same day each week.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Scale proportionally for larger vials (20 mg + 4 mL, 30 mg + 6 mL — same concentration). Draw: 0.25 mg = 5 u , 0.5 mg = 10 u, 1.0 mg = 20 u, 1.7 mg = 34 u, 2.4 mg = 48 u. Refrigerated, use within 28 days.
Cycling	Run continuously while losing; provider manages a maintenance dose or taper.
Stacks well with	Cagrilintide (CagriSema), Tesamorelin or CJC/Ipamorelin to protect muscle, B12 for energy.
Talking points	“The well-known weekly shot. It works by making you feel full sooner and longer. Works best with protein and resistance training so you lose fat, not muscle.”
Cautions	Nausea/constipation early (titrate slowly). Not with personal/family history of medullary thyroid cancer or MEN2. Provider screens. Protect muscle with protein + training.

Tirzepatide (GLP-1 / GIP)

YPB.203–208 (10–60 mg)

A dual agonist hitting both GLP-1 and GIP receptors (the Mounjaro/Zepbound molecule). The second receptor adds metabolic and appetite effects, so weight loss is typically greater than semaglutide for many people.

Benefits	Greater average weight loss than GLP-1 alone • Appetite + metabolic effect • Once-weekly • Strong blood-sugar control
Protocol	Start 2.5 mg SubQ weekly × 4 weeks, titrate by 2.5 mg every 4 weeks toward 10–15 mg as tolerated.
Reconstitution	10 mg vial + 1 mL BAC water = 10 mg/mL. (For larger vials: 20 mg + 2 mL, 30 mg + 3 mL, etc — same concentration.) Draw: 2.5 mg = 25 u, 5 mg = 50 u, 7.5 mg = 75 u, 10 mg = 100 u (full syringe) . For 12.5 / 15 mg, split into two injections of equal volume. Refrigerated, use within 28 days.

Cycling	Continuous during loss; provider manages maintenance.
Stacks well with	Same supportive stacks as semaglutide; do not combine with another GLP agonist.
Talking points	“The dual-action upgrade — it works on two appetite/metabolism pathways instead of one, so many people see more loss. Same easy weekly shot.”
Cautions	Same class cautions as semaglutide; GI side effects can be a bit stronger — titrate slowly.

Retatrutide (GLP-1 / GIP / Glucagon)

YPB.209–210, 234–236, 287

A triple agonist — GLP-1, GIP, and glucagon receptors. The glucagon arm adds energy expenditure/fat burning on top of appetite control. Investigational and the most powerful of the GLP class; reserved for aggressive goals under close provider guidance.

Benefits	Highest weight-loss potential of the class • Adds energy expenditure • Once-weekly
Protocol	Start 1–2 mg SubQ weekly, titrate slowly toward 8–12 mg under provider direction.
Reconstitution	20 mg vial + 2 mL BAC water = 10 mg/mL. (30 mg + 3 mL, 40 mg + 4 mL, 50 mg + 5 mL — same concentration.) Draw: 2 mg = 20 u, 4 mg = 40 u, 6 mg = 60 u, 8 mg = 80 u, 10 mg = 100 u. For 12 mg, split into two 60-unit injections. Refrigerated, use within 28 days.
Cycling	Continuous; close monitoring.
Stacks well with	Generally run solo within the GLP class.
Talking points	“The newest, strongest option — three pathways. Because it’s so potent we go slow and the provider watches it closely.”
Cautions	Most potent of the class, so the most GI/appetite effect. Investigational. Provider-directed only; slow titration.

Cagrilintide

YPB.239–241

A long-acting amylin analog. Amylin is a second satiety hormone (released with insulin). It complements GLP-1 drugs — the famous “CagriSema” pairing combines cagrilintide with semaglutide for additive appetite control.

Benefits	Added satiety • Synergistic with GLP-1 • Slows gastric emptying
Protocol	Weekly SubQ, titrated; commonly run with semaglutide. Provider sets ratio/dose.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 0.16 mg = 3 u, 0.3 mg = 6 u, 0.6 mg = 12 u, 1.2 mg = 24 u, 2.4 mg = 48 u. Refrigerated, use within 28 days.
Stacks well with	Semaglutide (CagriSema).
Talking points	“A different fullness hormone — stacking it with semaglutide hits appetite from two angles.”
Cautions	GI effects; same slow-titration logic. Provider-directed combination.

Mazdutide & Survodutide

YPB.269, YPB.278

Both are GLP-1 / glucagon dual agonists (investigational). Similar idea to tirzepatide but with a glucagon arm instead of GIP — appetite control plus increased energy expenditure and potential liver-fat benefits.

Benefits	Appetite + energy expenditure • Emerging liver-fat data • Weekly
Protocol	Titrated weekly SubQ, provider-directed.
Reconstitution	Standard recipe: vial + enough BAC water to reach 5 mg/mL (e.g., 10 mg + 2 mL). Investigational — provider sets the dose-to-units conversion based on the actual vial label.
Talking points	“Newer dual-action options in the same family — we’d let the provider decide if they fit better than the mainstream choices.”
Cautions	Investigational; provider-directed; class GI effects.

AOD-9604

YPB.248

A modified fragment of growth hormone (the 176–191 region) that targets fat metabolism (lipolysis) *without* the blood-sugar or growth effects of full GH. A gentler, non-GLP fat-loss aid.

Benefits	Stimulates fat breakdown • No GH-style side effects • Good for GLP-averse customers
Protocol	~300 mcg SubQ daily, often fasted/morning.
Reconstitution	2 mg vial + 2 mL BAC water = 1 mg/mL (1,000 mcg/mL) . Draw: 300 mcg = 30 u , 250 mcg = 25 u, 500 mcg = 50 u. Refrigerated, use within 21–28 days.
Stacks well with	GLP drugs, MIC/lipotropics, GH stacks.
Talking points	“A gentle, non-GLP fat-metabolism helper — a fragment of GH that just tells fat cells to release stored fat.”
Cautions	Mild; effects are subtle vs. GLP drugs — set expectations.

5-Amino-1MQ & SLU-PP-332

YPB.242, 247, 243

5-Amino-1MQ is an oral NNMT inhibitor that supports fat metabolism and lean mass. **SLU-PP-332** is an “exercise-mimetic” (ERR agonist) that boosts fat oxidation and endurance — like signaling “you just worked out.”

Benefits	Fat metabolism support • 5-Amino-1MQ is oral (no injection) • Endurance / exercise-mimetic
Protocol	5-Amino-1MQ: oral capsule daily. SLU-PP-332: SubQ, provider-directed.
Reconstitution	5-Amino-1MQ : oral capsule, no reconstitution. SLU-PP-332 : research compound — provider sets the recipe per the vial label.
Talking points	“For customers who want metabolic support without a shot, 5-Amino-1MQ is an oral option.”
Cautions	Research compounds; subtle effects; adjuncts not standalone solutions.

HEALING & TISSUE REPAIR

The recovery toolkit — repair tissue, calm inflammation, and improve gut and skin integrity. BPC-157 and TB-500 are the workhorses; the copper peptides bridge into skincare.

BPC-157

YPB.212/213/237 (5/10/20 mg)

“Body Protective Compound” — a peptide derived from a protein in stomach acid. It promotes angiogenesis (new blood-vessel growth) to speed healing of tendons, ligaments, muscle, and especially the gut lining. The single most popular healing peptide.

Benefits	Tendon/ligament/muscle repair • Gut healing (ulcers, leaky gut, IBD-type issues) • Reduces inflammation • Protects the gut from NSAIDs
Protocol	250–500 mcg SubQ 1–2×/day, often injected near the injury site. Courses of 4–8 weeks.
Reconstitution	5 mg vial + 2 mL BAC water = 2.5 mg/mL (2,500 mcg/mL). (10 mg vial + 4 mL, 20 mg + 8 mL — same concentration.) Draw: 250 mcg = 10 u, 500 mcg = 20 u. Refrigerated, use within 14–21 days. For a tighter draw, alt recipe is 5 mg + 1 mL = 5 mg/mL (250 mcg = 5 u, 500 mcg = 10 u).
Cycling	Run through the injury/healing window, then stop.
Stacks well with	TB-500 (“Wolverine”), GHK-Cu, GH stack for connective tissue.
Talking points	“The go-to healing peptide — great for nagging tendon/joint injuries and for gut health. People often inject it close to the sore area.”
Cautions	Well-tolerated. Banned in competitive sport. Quality/source matters — ours is Alpha BioMed.

TB-500 (Thymosin Beta-4 fragment)

YPB.214/215 (5/10 mg)

A synthetic version of a region of Thymosin Beta-4. It regulates actin (a cell-building protein), improving cell migration to injury sites — so it boosts flexibility, reduces scar tissue, and aids systemic, whole-body healing.

Benefits	Whole-body / systemic healing • Flexibility & reduced adhesions • Muscle & tendon repair
Protocol	~2–2.5 mg SubQ twice weekly as a loading phase (4–6 weeks), then a maintenance dose.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. Draw: 2 mg = 40 u, 2.5 mg = 50 u, 4 mg = 80 u. (Or 5 mg vial + 2 mL = 2.5 mg/mL: 2 mg = 80 u — same dose, more volume.) Refrigerated, use within 14–21 days.
Cycling	Loading phase then maintenance; cycle off when healed.
Stacks well with	BPC-157 (the classic recovery duo).
Talking points	“The systemic partner to BPC — works body-wide on flexibility and recovery, where BPC is great for a specific spot.”
Cautions	Banned in sport. Loading dose then taper.

Wolverine Blend (BPC-157 + TB-500)

YPB.216/217 (5/10 mg)

A pre-combined blend of the two recovery workhorses in one vial — local + systemic healing together. Named for the comic-book fast-healing factor. The easiest single-product recovery answer.

Benefits	Combined local + systemic healing • One injection instead of two • Faster recovery
Protocol	Dosed per the blend ratio, typically daily-to-several-times-weekly for 4–6 weeks.
Reconstitution	5 mg blend vial + 2 mL BAC water (10 mg + 4 mL — same concentration). The per-component split depends on the blend ratio printed on the vial — always check the label before drawing. Daily to several-times-weekly dosing in 4–6 week cycles.
Talking points	“Both healing peptides in one shot — the convenient, complete recovery option.”
Cautions	Banned in sport; same as components.

GHK-Cu (Copper Peptide)

YPB.221/222 (50/100 mg)

A naturally occurring copper-binding tripeptide. A powerhouse for skin and tissue: it drives collagen and elastin production, wound healing, hair-follicle stimulation, and has antioxidant/anti-inflammatory effects. Used both injected and topically.

Benefits	Collagen & elastin (firmer skin) • Wound & scar healing • Hair growth • Antioxidant / anti-inflammatory
Protocol	Topical serum daily for skin; low-dose SubQ for systemic skin/tissue benefits. Provider-directed for injection.
Reconstitution	50 mg vial + 5 mL BAC water = 10 mg/mL. (100 mg vial + 10 mL — same concentration.) Draw for systemic SubQ: 1 mg = 10 u, 2 mg = 20 u. Topical: apply a few drops of reconstituted solution to skin. Refrigerated, use within 21 days.
Stacks well with	BPC/TB (GLOW, KLOW blends), Glutathione, Snap-8 topically.
Talking points	“The copper peptide — the star of our skincare line. Builds collagen, heals skin, even helps hair. Works as a serum or an injection.”
Cautions	Topical can tint blue-green (it's copper). Avoid pairing topical with strong acids/retinoids at the same time.

GLOW Blend (GHK-Cu + BPC + TB)

YPB.218

An “inside-out” beauty + healing blend: the copper peptide for skin combined with the two healing peptides. Marketed for skin glow, recovery, and overall tissue quality in one product.

Benefits	Skin glow + collagen • Whole-body healing • Convenient all-in-one
Protocol	Per-blend dosing, typically daily SubQ in cycles.
Reconstitution	Blend vial + 2 mL BAC water. Per-component concentrations depend on the blend ratio on the label — check it before drawing. Refrigerated, use within 14 days.
Talking points	“Our signature glow stack — skin, hair, and healing in one vial.”
Cautions	Combination of three; introduce when client tolerates components.

KLOW Blend (GHK-Cu + KPV + BPC + TB)

YPB.264

GLOW plus KPV, an anti-inflammatory tripeptide. The added KPV makes it the choice for reactive, inflamed, or gut-linked skin issues — comprehensive skin + recovery + calm.

Benefits	Everything GLOW does • Adds anti-inflammatory KPV • Best for irritated/reactive skin
Protocol	Per-blend dosing, daily SubQ in cycles.
Reconstitution	Blend vial + 2 mL BAC water. Four-component blend — per-mL split is on the vial label. Refrigerated, use within 14 days.
Talking points	“The upgraded glow blend — adds an anti-inflammatory peptide, so it’s ideal if skin is reactive or there’s a gut-skin connection.”
Cautions	Four-component blend; provider-directed.

LL-37

YPB.244

The body’s own antimicrobial peptide (a cathelicidin). It fights microbes, modulates immune response, and aids wound healing — used in research for chronic infection, gut, and skin contexts.

Benefits	Antimicrobial • Immune modulation • Wound healing
Protocol	Low-dose SubQ, provider-directed cycles.
Reconstitution	5 mg vial + 2 mL BAC water = 2.5 mg/mL (2,500 mcg/mL). Draw: 100 mcg = 4 u, 250 mcg = 10 u, 500 mcg = 20 u. Refrigerated, use within 14 days.
Talking points	“A natural antimicrobial peptide — used for stubborn infections and immune/gut support.”
Cautions	Can transiently provoke immune response; provider-directed.

GROWTH HORMONE (Secretagogues & GHRH)

Instead of injecting synthetic HGH, these prompt the body to release *its own* growth hormone — for muscle, fat loss, recovery, sleep, and anti-aging. The classic move is to pair a GHRH (tells the pituitary to make GH) with a secretagogue (tells it to release a pulse).

Sermorelin

YPB.211 (10 mg)

A GHRH analog — the foundational, gentle GH stimulator. It nudges the pituitary to produce and release GH in a natural pulsatile pattern, so levels stay physiologic. A common first GH-optimization product.

Benefits	Natural GH increase • Better sleep • Recovery, body composition, skin
Protocol	~200–300 mcg SubQ nightly before bed (GH releases during deep sleep).
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 200 mcg = 4 u, 300 mcg = 6 u. For a more precise small-dose draw, alt recipe is 10 mg + 5 mL = 2 mg/mL (200 mcg = 10 u, 300 mcg = 15 u). Refrigerated, use within 28 days.
Cycling	Often run 5 days on / 2 off, in multi-month cycles.
Stacks well with	Ipamorelin (GHRH + secretagogue synergy).
Talking points	“The gentle entry to GH optimization — helps your own GH come back up. Most people notice sleep and recovery first.”
Cautions	Inject on an empty stomach (food/carbs blunt the GH pulse). Mild water retention possible.

CJC-1295 (with & without DAC)

YPB.219, YPB.220

A more potent GHRH analog. **With DAC** it's long-acting (steady GH elevation, dose ~weekly). **Without DAC** (“Mod GRF 1-29”) is short-acting for clean GH pulses, usually stacked with Ipamorelin and dosed daily.

Benefits	Stronger GH stimulation than Sermorelin • Flexible (steady vs. pulsatile) • Muscle, fat loss, recovery
Protocol	No-DAC: ~100 mcg SubQ with Ipamorelin, 1–2×/day before bed/fasted. With-DAC: weekly dosing.
Reconstitution	Without DAC (Mod GRF 1-29): 2 mg vial + 2 mL BAC = 1 mg/mL (1,000 mcg/mL). Draw: 100 mcg = 10 u, 200 mcg = 20 u, 300 mcg = 30 u. With DAC: 2 mg vial + 1 mL BAC = 2 mg/mL. Draw: 1 mg = 50 u, 2 mg = 100 u. Refrigerated, use within 21–28 days.
Stacks well with	Ipamorelin — the gold-standard GH pair.
Talking points	“A stronger GH-releasing peptide. We almost always pair the daily version with Ipamorelin for a clean GH pulse.”
Cautions	Fasted dosing for pulse versions. Water retention/tingling possible early.

Ipamorelin

YPB.263 (10 mg)

A selective GH secretagogue (a ghrelin-receptor mimic). It triggers a clean GH pulse *without* raising cortisol, prolactin, or hunger — which is why it's the preferred secretagogue and the standard partner for CJC-1295.

Benefits	Clean GH pulse (no cortisol/prolactin spike) • Recovery, sleep, lean mass • Well-tolerated
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Protocol	~200–300 mcg SubQ, 1–3×/day (commonly before bed), paired with CJC-1295.
Reconstitution	10 mg vial + 5 mL BAC water = 2 mg/mL (2,000 mcg/mL). Draw: 100 mcg = 5 u, 200 mcg = 10 u, 300 mcg = 15 u. Refrigerated, use within 28 days. (Higher-concentration alt: 10 mg + 2 mL = 5 mg/mL — but units get too small to read precisely.)
Stacks well with	CJC-1295 (the classic stack), Sermorelin.
Talking points	“The clean GH-release peptide — no hunger or cortisol spike. It’s the partner to CJC for the best GH effect.”
Cautions	Very well-tolerated. Fasted dosing maximizes the pulse.

Tesamorelin

YPB.279/288 (10/20 mg)

A potent GHRH analog that is FDA-approved (as Egrifta) specifically to reduce **visceral (deep belly) fat**. The standout GH peptide when the goal is targeting stubborn abdominal fat.

Benefits	Targets visceral belly fat • Raises GH/IGF-1 • Cognitive & metabolic benefits
Protocol	~1–2 mg SubQ daily (often at night).
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. (20 mg + 4 mL — same concentration.) Draw: 1 mg = 20 u, 2 mg = 40 u. Refrigerated, use within 28 days.
Stacks well with	Ipamorelin; pairs with GLP weight-loss protocols to protect muscle.
Talking points	“The one specifically known for shrinking deep belly fat — it’s even FDA-approved for that purpose. Great alongside a weight-loss plan.”
Cautions	Daily injection; can affect blood sugar — provider monitors.

IGF-1 LR3 & IGF-DES

YPB.262/285 (1/0.1 mg),
YPB.286

GH works largely *through* IGF-1. **IGF-1 LR3** is a long-acting form that directly signals muscle growth and nutrient partitioning. **IGF-DES** is a variant favored for localized muscle effect. Advanced, potent — used by experienced clients.

Benefits	Direct muscle growth signaling • Nutrient partitioning • Localized growth (DES)
Protocol	Low microgram doses SubQ, often post-workout; advanced/provider-directed.
Reconstitution	1 mg vial + 1 mL BAC water = 1 mg/mL (1,000 mcg/mL). Draw: 20 mcg = 2 u, 50 mcg = 5 u, 100 mcg = 10 u. Refrigerated, use within 14–21 days. <i>Advanced — provider-directed only.</i>
Stacks well with	GH-releasing stacks; not for beginners.
Talking points	“The advanced muscle-builder — it’s what GH works through. We’d only set this up with the provider for experienced users.”
Cautions	Can drop blood sugar (hypoglycemia risk). Advanced only; provider-directed.

GHRP-6 & Hexarelin

YPB.257/282, YPB.261

Older, potent GH secretagogues. **GHRP-6** strongly stimulates GH and also appetite (useful for hard-gainers). **Hexarelin** is one of the most potent GH releasers. Both predate Ipamorelin, which is now preferred for being cleaner.

Benefits	Strong GH release • GHRP-6 boosts appetite • Recovery/mass
Protocol	~100 mcg SubQ, fasted; provider-directed.
Reconstitution	5 mg vial + 2 mL BAC water = 2.5 mg/mL (2,500 mcg/mL). Draw: 100 mcg = 4 u, 200 mcg = 8 u, 300 mcg = 12 u. Refrigerated, use within 21 days.
Talking points	“Stronger but older GH-release peptides — GHRP-6 also bumps appetite if someone struggles to eat enough.”
Cautions	Can raise cortisol/prolactin and cause hunger; Ipamorelin is the cleaner default.

DSIP & GDF-8

YPB.230/252 (15/5 mg),
YPB.233

DSIP (Delta Sleep-Inducing Peptide) promotes deep sleep and supports the GH that releases during it. **GDF-8** relates to myostatin (the muscle-limiting protein) and is a research compound in muscle work.

Benefits	DSIP: deeper sleep + GH support • Recovery • GDF-8: myostatin research
Protocol	DSIP: low dose SubQ before bed. GDF-8: research, provider-directed.
Reconstitution	DSIP 15 mg vial + 3 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 100 mcg = 2 u, 200 mcg = 4 u, 300 mcg = 6 u. (5 mg vial + 1 mL = same 5 mg/mL.) GDF-8 is research-grade — provider sets recipe.
Talking points	“DSIP is our sleep-and-recovery peptide — deep sleep is when GH does its work.”
Cautions	GDF-8 is research-grade; provider-directed.

SEXUAL HEALTH

Libido and arousal peptides for men and women. The standout is PT-141, which works centrally (in the brain) rather than on blood flow — so it helps when traditional ED drugs don't.

PT-141 (Bremelanotide)

YPB.274 (10 mg)

A melanocortin-receptor agonist that increases sexual desire and arousal by acting on the **brain's arousal pathways** — not blood vessels. That central mechanism means it works for both men and women, and for people whose issue is desire, not just erection.

Benefits	Increases libido/desire (men & women) • Works centrally, not on blood flow • Taken as-needed before activity
Protocol	~1–2 mg SubQ about 45 minutes before intimacy. Not daily — as-needed. Start low.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 1 mg = 20 u, 1.5 mg = 30 u, 1.75 mg = 35 u. Use ~45 min before intimacy. Refrigerated, use within 21 days.
Stacks well with	Provider TRT/HCG protocols for men.
Talking points	"It works on the desire centers in the brain, so it helps when the issue is wanting to, not just the physical side — and it works for women too."
Cautions	Nausea and facial flushing are dose-related — start low. Can transiently raise blood pressure. Provider screens cardiovascular history.

Kisspeptin-10

YPB.266 (10 mg)

An upstream signaling peptide that stimulates GnRH release, driving LH and FSH — the body's natural sex-hormone cascade. Researched for libido, fertility, and a more "natural-axis" approach to hormone support.

Benefits	Stimulates natural sex-hormone production • Libido • Fertility signaling
Protocol	SubQ, provider-directed dosing.
Reconstitution	10 mg vial + 5 mL BAC water = 2 mg/mL (2,000 mcg/mL). Draw: 100 mcg = 5 u, 200 mcg = 10 u, 400 mcg = 20 u. Refrigerated, use within 21 days.
Talking points	"This one works upstream — it tells the body to make its own sex hormones, a more natural-axis approach."
Cautions	Provider-directed; effects are signaling-based and individual.

Melanotan 2

YPB.270 (10 mg)

A melanocortin agonist best known for stimulating melanin (tanning) with minimal sun. It shares PT-141's family, so it also carries a libido side-benefit. Popular for tanning, especially in fair-skinned customers.

Benefits	Sunless/accelerated tanning • Libido side-benefit • Possible appetite reduction
Protocol	Low-dose SubQ loading then maintenance; provider-directed.

Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: start 250 mcg = 5 u, 500 mcg = 10 u, 1 mg = 20 u. Refrigerated, use within 28 days.
Talking points	“The tanning peptide — builds a tan with less sun, and some people notice a libido bump too.”
Cautions	Can darken moles/freckles and cause nausea. Advise a skin/mole check first. Not a substitute for sun protection.

COGNITIVE & NEURO

Focus, mood, anxiety, and neuroprotection. The Russian nootropics Semax and Selank lead this group — most are intranasal because they target the brain.

Semax

YPB.229 (10 mg)

A nootropic peptide (an ACTH fragment) that boosts focus, mental energy, and learning by raising BDNF (brain-derived neurotrophic factor) and modulating neurotransmitters. Also studied for neuroprotection and stroke recovery. Usually intranasal.

Benefits	Focus & mental clarity • Raises BDNF (neuroplasticity) • Neuroprotective
Protocol	Intranasal spray, mornings/daytime, as directed.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Intranasal: usually 2–4 drops/spray per nostril per dose (per the label). SubQ draw if injecting: 200 mcg = 4 u, 500 mcg = 10 u, 1 mg = 20 u. Refrigerated, use within 14 days.
Stacks well with	Selank (the “focused but calm” daytime pair).
Talking points	“The focus-and-clarity nasal spray — like a clean mental boost, and it supports brain health long-term.”
Cautions	Well-tolerated; intranasal.

Selank

YPB.228 (10 mg)

An anxiolytic nootropic (derived from a natural immune peptide, tuftsin). It reduces anxiety and stabilizes mood *without* sedation or dependence — calm and clear rather than drowsy. Intranasal.

Benefits	Reduces anxiety • Calm focus, no sedation • Non-habit-forming
Protocol	Intranasal spray as needed for calm focus.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Intranasal: 2–4 drops/spray per nostril per dose (per the label). SubQ: 250 mcg = 5 u, 500 mcg = 10 u, 1 mg = 20 u. Refrigerated, use within 14 days.
Stacks well with	Semax.
Talking points	“The calm nasal spray — takes the edge off anxiety without making you tired or foggy.”
Cautions	Well-tolerated; intranasal.

SS-31 (Elamipretide)

YPB.245/246 (10/50 mg)

A mitochondria-targeted peptide that protects the cell's energy factories from oxidative damage, restoring cellular energy. Researched for fatigue, neuro/cardiac health, and aging — wherever poor mitochondrial function shows up as low energy or brain fog.

Benefits	Mitochondrial protection • Cellular energy • Anti-fatigue / anti-aging
Protocol	SubQ daily in cycles, provider-directed.

Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. Draw: 1 mg = 20 u, 2 mg = 40 u, 3 mg = 60 u. (50 mg vial + 5 mL = 10 mg/mL: 1 mg = 10 u, 5 mg = 50 u.) Refrigerated, use within 14 days, protect from light.
Stacks well with	NAD+, MOTS-c (mitochondrial/longevity).
Talking points	“It protects and recharges your cells’ energy factories — for people running on empty or with brain fog.”
Cautions	Provider-directed; cyclical.

Pinealon

YPB.273 (20 mg)

A short peptide bioregulator that targets brain/neuronal tissue — researched for cognitive function, neuroprotection, and circadian/sleep regulation. Part of the “bioregulator” longevity family.

Benefits	Cognitive support • Neuroprotection • Circadian/sleep support
Protocol	Short SubQ cycles (e.g., 10–20 days), provider-directed.
Reconstitution	20 mg vial + 2 mL BAC water = 10 mg/mL. Draw: 5 mg = 50 u, 10 mg = 100 u. Refrigerated, use within 21 days.
Talking points	“A brain-targeted bioregulator — run in short cycles for cognition and sleep rhythm.”
Cautions	Cyclical; provider-directed.

ANTI-AGING & LONGEVITY

The longevity stack — cellular energy, DNA/telomere maintenance, and clearing aged cells. NAD+ and Epitalon are the cornerstones; FOXO4-DRI is the advanced senolytic.

NAD+

YPB.223/224 (500/1000 mg)

NAD+ is a coenzyme in every cell, essential for energy production and DNA repair — and it declines with age. Supplementing supports cellular energy, repair, metabolism, and mental clarity. The cornerstone longevity molecule.

Benefits	Cellular energy • DNA repair • Mental clarity • Metabolic support
Protocol	SubQ — titrate dose slowly; injecting too fast causes flushing/discomfort. Often smaller daily or larger weekly dosing.
Reconstitution	500 mg vial + 5 mL sterile saline (NOT bacteriostatic) = 100 mg/mL. (1,000 mg + 10 mL — same concentration.) Inject slowly — fast injection causes flushing/chest tightness. Draw: 50 mg = 50 u, 100 mg = 100 u (full syringe). Often prepared by a compounding pharmacy — check label.
Stacks well with	Glutathione, MOTS-c, Epitalon, GH stacks.
Talking points	“The foundational anti-aging molecule — it powers your cells and repairs DNA. Go slow with the dose; rushing it just causes flushing.”
Cautions	Inject slowly — fast dosing causes a wave of flushing/chest tightness (harmless but uncomfortable).

Epitalon (Epithalon)

YPB.232/253/254 (5/10/50 mg)

A peptide bioregulator of the pineal gland that has been shown to activate telomerase (the enzyme that maintains telomeres, the protective caps on DNA). Researched for longevity, sleep, and circadian regulation. Run in short cycles a few times a year.

Benefits	Telomere/telomerase support • Better sleep & circadian rhythm • Longevity / anti-aging
Protocol	Short SubQ cycle (e.g., 10–20 days), repeated 2–4×/year.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). (5 mg + 1 mL = same; 50 mg + 10 mL = same.) Draw: 1 mg = 20 u, 2.5 mg = 50 u, 5 mg = 100 u (full syringe) . For 10 mg dose, split into two 5 mg injections. Refrigerated, use within 21 days.
Stacks well with	NAD+, MOTS-c.
Talking points	“The telomere peptide — run as a short cycle a few times a year for longevity and better sleep.”
Cautions	Cyclical, not continuous; provider-directed.

MOTS-c

YPB.227/271 (10/40 mg)

A mitochondrial-derived peptide that acts like an “exercise mimetic” — improving insulin sensitivity, fat metabolism, and endurance by signaling through metabolic pathways. Bridges longevity and performance/weight goals.

Benefits	Metabolic health / insulin sensitivity • Endurance (exercise-mimetic) • Fat metabolism
Protocol	SubQ several times weekly in cycles; provider-directed.
Reconstitution	10 mg vial + 1 mL BAC water = 10 mg/mL. Draw: 2.5 mg = 25 u, 5 mg = 50 u, 10 mg = 100 u. (40 mg + 4 mL — same concentration.) Refrigerated, use within 21 days.
Stacks well with	NAD+, SS-31, weight/performance protocols.
Talking points	“A mitochondrial peptide that mimics some benefits of exercise — metabolism and endurance.”
Cautions	Cyclical; provider-directed.

FOXO4-DRI

YPB.255 (10 mg)

An advanced **senolytic** — it selectively triggers the death of senescent (“zombie”) cells that accumulate with age and drive inflammation. Used in short, infrequent cycles. One of the most cutting-edge longevity research peptides.

Benefits	Clears senescent cells • Reduces age-related inflammation • Cutting-edge longevity
Protocol	Short, infrequent cycles; advanced, provider-directed.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. Advanced senolytic — provider sets the actual dose. Short, infrequent cycles only. Refrigerated, use within 14 days.
Talking points	“The ‘zombie-cell’ clearer — it removes worn-out cells that drive aging. Very cutting-edge, used in short cycles.”
Cautions	Advanced/experimental; provider-directed only; infrequent cycling.

PNC-27

YPB.275 (10 mg)

A research peptide studied for its ability to selectively target cancer-cell membranes (via an HDM-2/p53 mechanism). **Strictly research-use** — staff should not make any therapeutic or anticancer claims; route all such questions to the provider.

Benefits	Research compound (oncology research context)
Protocol	Research-use; provider-directed only.
Reconstitution	Research compound. Provider sets the recipe per vial label. Staff make no therapeutic claims and route all questions to the provider.
Talking points	“This is a research peptide. I can’t make any medical claims about it — the provider is the right person for any questions here.”
Cautions	Make NO therapeutic/anticancer claims. Research-use only. Always route to provider.

SLEEP, RECOVERY & NEURO-SUPPORT

Deep sleep, nervous-system recovery, and nerve/inflammation support — the peptides that help the body repair when it's at rest.

DSIP (Delta Sleep-Inducing Peptide)

YPB.230/252 (15/5 mg)

A naturally occurring peptide that promotes delta-wave (deep) sleep, helps regulate the sleep-wake cycle, and reduces stress. Because deep sleep is when the body recovers and releases GH, it's a recovery peptide as much as a sleep aid.

Benefits	Deeper sleep • Stress reduction • Supports nighttime GH/recovery
Protocol	Low dose SubQ before bed.
Reconstitution	15 mg vial + 3 mL BAC water = 5 mg/mL (5,000 mcg/mL). (5 mg + 1 mL = same.) Draw: 100 mcg = 2 u, 200 mcg = 4 u, 300 mcg = 6 u. Refrigerated, use within 21 days.
Stacks well with	GH stack (Sermorelin/CJC/Ipamorelin), Epitalon.
Talking points	"For deep, restorative sleep — and since recovery happens during sleep, it helps you bounce back too."
Cautions	Timing matters (before bed); individual response varies.

VIP (VIP10)

YPB.281 (10 mg)

Vasoactive Intestinal Peptide — a signaling molecule with broad anti-inflammatory and immune-regulating roles. Researched for chronic inflammatory conditions (including mold/CIRS protocols) and gut/respiratory health. Usually intranasal.

Benefits	Anti-inflammatory • Immune regulation • Used in CIRS/chronic-inflammation protocols
Protocol	Intranasal, provider-directed (often after other CIRS steps).
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. Usually delivered intranasally — drops/spray dosing per the vial label. Refrigerated, use within 14 days.
Talking points	"An anti-inflammatory signaling peptide — used in chronic-inflammation and mold-illness protocols, usually under provider guidance."
Cautions	Can transiently affect blood pressure; provider-directed, often later-stage in a protocol.

ARA-290 (Cibinetide)

YPB.277 (10 mg)

A peptide derived from erythropoietin that targets the innate repair receptor — researched for neuropathic pain, nerve repair, and reducing inflammation, without EPO's blood effects.

Benefits	Neuropathic pain relief • Nerve repair • Anti-inflammatory
Protocol	SubQ daily in cycles; provider-directed.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 1 mg = 20 u, 2 mg = 40 u, 4 mg = 80 u. Refrigerated, use within 21 days.

Talking points	“For nerve pain and nerve repair — it calms inflammation around nerves.”
Cautions	Provider-directed; cyclical.

IMMUNE SUPPORT

Immune modulation and thymic function — peptides that help balance and strengthen the immune system rather than just stimulate it.

Thymosin Alpha-1 (TA1)

YPB.231 (10 mg)

A thymic peptide that modulates and strengthens the immune system — enhancing T-cell function and balancing immune response. Used clinically worldwide for infections and immune support. The headline immune peptide.

Benefits	Strengthens immune defense • Balances/modulates immune response • Supports recovery from illness
Protocol	SubQ, 1–2×/week or daily in acute settings; provider-directed.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). (5 mg vial + 1 mL = same concentration.) Draw: 900 mcg = 18 u, 1.5 mg = 30 u, 1.6 mg = 32 u. Refrigerated, use within 21 days.
Stacks well with	Thymalin, glutathione.
Talking points	“The immune-support peptide — it tunes up and balances your immune system, great for resilience and recovering from being run-down.”
Cautions	Generally very safe; provider-directed in autoimmune contexts.

Thymalin

YPB.280 (10 mg)

A thymic peptide bioregulator that supports and restores thymus function (the thymus drives T-cell immunity and shrinks with age). Run in short cycles for immune restoration and anti-aging of the immune system.

Benefits	Restores thymic/immune function • Immune anti-aging • Cyclical
Protocol	Short SubQ cycles (e.g., 10 days), repeated; provider-directed.
Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL. Draw: 2.5 mg = 50 u, 5 mg = 100 u (full syringe). Refrigerated, use within 14 days; run in short cycles.
Talking points	“Rejuvenates the thymus — the gland that runs your immune system and fades with age. Run in short cycles.”
Cautions	Cyclical; provider-directed.

KPV

YPB.265 (10 mg)

A tripeptide fragment of alpha-MSH with potent anti-inflammatory action, especially in the gut and skin. Used for inflammatory bowel issues, mast-cell/allergic inflammation, and as the calming component in the KLOW skin blend.

Benefits	Anti-inflammatory (gut & skin) • Helps IBD/mast-cell issues • Non-immunosuppressive
Protocol	Oral/SubQ depending on target; provider-directed.

Reconstitution	10 mg vial + 2 mL BAC water = 5 mg/mL (5,000 mcg/mL). Draw: 250 mcg = 5 u, 500 mcg = 10 u, 1 mg = 20 u. Refrigerated, use within 21 days.
Stacks well with	BPC-157 (gut), KLOW blend (skin).
Talking points	“A targeted anti-inflammatory — especially for gut and skin inflammation, without shutting down the immune system.”
Cautions	Provider-directed; well-tolerated.

BODY COMPOSITION & HORMONE SUPPORT

Muscle preservation/building and hormone-axis support — including the fertility and TRT-adjacent peptides that often come up in men's-health conversations.

HCG (Human Chorionic Gonadotropin)

YPB.256 (10,000 iu)

Mimics LH to stimulate the testes (testosterone & sperm) or support ovulation. In men's health it's used alongside TRT to maintain natural testicular function and fertility. Also appears in some medically-supervised weight protocols.

Benefits	Maintains testicular function on TRT • Supports fertility • Hormone-axis support
Protocol	IM/SubQ, provider-directed dosing (often 2–3×/week with TRT).
Reconstitution	10,000 iu vial + 10 mL bacteriostatic water (or saline) = 1,000 iu/mL. Draw: 250 iu = 25 u, 500 iu = 50 u, 1,000 iu = 100 u. (Alt: + 5 mL = 2,000 iu/mL: 500 iu = 25 u, 1,000 iu = 50 u.) Refrigerated, use within 28 days. <i>Provider-directed.</i>
Stacks well with	Provider TRT protocols, HMG for fertility.
Talking points	"Often used with testosterone therapy to keep the body's own production and fertility going. This is firmly a provider-managed protocol."
Cautions	Hormone-axis drug — provider-directed only. Route all TRT/fertility questions to the provider.

HMG (Human Menopausal Gonadotropin)

YPB.258 (75 iu)

Provides FSH (and LH) activity to drive sperm production in men or follicle development in women — a fertility-focused tool, often paired with HCG.

Benefits	Fertility support (FSH activity) • Sperm/follicle stimulation
Protocol	IM/SubQ, provider-directed.
Reconstitution	75 iu vial + 1 mL saline or BAC water = 75 iu/mL. Typical single-vial dose draws the full 100 u. Provider-directed.
Stacks well with	HCG.
Talking points	"A fertility-support hormone — usually managed with HCG by the provider."
Cautions	Fertility/hormone drug — provider-directed only.

ACE-031

YPB.249 (1 mg)

A myostatin/activin inhibitor (a decoy receptor). By blocking myostatin — the protein that *limits* muscle growth — it's researched for increasing muscle mass and strength. Advanced research compound.

Benefits	Increases muscle mass (myostatin inhibition) • Strength • Research compound
Protocol	Research-use; provider-directed.

Reconstitution	1 mg vial + 1 mL BAC water = 1 mg/mL (1,000 mcg/mL). Draw: 100 mcg = 10 u, 250 mcg = 25 u, 500 mcg = 50 u. Refrigerated, use within 14 days. <i>Research compound — provider sets the dose.</i>
Talking points	“A research compound that blocks the body’s muscle-limiting signal — advanced, provider-only.”
Cautions	Research-grade; can affect blood vessels in studies; provider-directed only.

AICAR

YPB.250 (50 mg)

An AMPK activator — it switches on the cellular energy sensor that’s normally triggered by exercise, boosting fat oxidation and endurance. An “exercise-mimetic” for body comp and stamina.

Benefits	Fat oxidation • Endurance • Exercise-mimetic (AMPK)
Protocol	SubQ, provider-directed.
Reconstitution	50 mg vial + 5 mL BAC water = 10 mg/mL. Draw: 10 mg = 100 u (full syringe). Larger research doses are typically split across multiple injections or delivered IV in clinic — provider-directed.
Stacks well with	SLU-PP-332, MOTS-c, weight protocols.
Talking points	“Flips on the same energy switch as exercise — supports fat burning and endurance.”
Cautions	Research compound; banned in sport; provider-directed.

VITAMINS & LIPOTROPICS

Injectable nutrients and fat-metabolism blends — the gentle, broadly-appealing add-ons that pair with almost any goal and are easy first purchases.

B12 (Methylcobalamin)

YPB.251 (10 ml)

Injectable vitamin B12 for energy, metabolism, nerve function, and red-blood-cell production. A simple, popular energy add-on — especially for customers in a calorie deficit on a weight protocol.

Benefits	Energy • Metabolism support • Nerve & blood-cell health
Protocol	IM/SubQ, weekly or as directed.
Reconstitution	Pre-mixed solution, typically 1,000 mcg/mL (1 mg/mL) . Draw: 500 mcg = 50 u, 1,000 mcg = 100 u (full syringe). No reconstitution needed — confirm concentration on the bottle label.
Stacks well with	Weight-loss protocols, MIC blends, glutathione.
Talking points	“The classic energy shot — really popular alongside weight loss to keep energy up.”
Cautions	Very safe, water-soluble.

Glutathione

YPB.259/283 (1500/600 mg)

The body's master antioxidant. It detoxifies, protects cells from oxidative stress, supports the liver and immune system, and is prized for skin brightening and an even tone. Bridges skincare, longevity, and recovery.

Benefits	Master antioxidant / detox • Skin brightening & even tone • Liver & immune support
Protocol	IM/SubQ (or IV in clinic), 1–2×/week.
Reconstitution	Often pre-mixed at 200 mg/mL. Draw: 100 mg = 50 u, 200 mg = 100 u. For dry powder: 1,500 mg vial + 7.5 mL BAC = 200 mg/mL. Provider/compounder sets the concentration on the actual product.
Stacks well with	Glutathione + B12 + NAD+ (the wellness trio); skincare stacks.
Talking points	“The master antioxidant — detox, immune support, and a real favorite for brighter, more even skin.”
Cautions	Very well-tolerated.

Lipotropic Blends (8X / 4X MIC)

YPB.267/268

MIC injections — **M**ethionine, **I**nositol, **C**holine — plus B vitamins. These lipotropic compounds help the liver mobilize and metabolize fat, and the B vitamins add energy. The 8X blend is the more comprehensive version. A staple fat-loss support shot.

Benefits	Supports fat metabolism • Energy (B vitamins) • Pairs with any weight protocol
Protocol	IM/SubQ, typically weekly.
Reconstitution	Pre-mixed solution — no reconstitution. Dosed in mL per the label; typical MIC injection is 0.5–1 mL IM/SubQ weekly.

Stacks well with	GLP weight-loss drugs, B12, AOD-9604.
Talking points	“The fat-burner support shot — helps your liver process fat and gives an energy boost. Great companion to a weight-loss plan.”
Cautions	Well-tolerated; an adjunct, not a standalone weight-loss solution.

SUPPLIES & THE BASICS

What every customer needs to use peptides safely. The #1 follow-up question after “which peptide” is “how do I actually mix and inject it” — own this answer.

Bacteriostatic / Reconstitution Water

YPB.225/226 (3/10 ml)

Sterile water with 0.9% benzyl alcohol that's used to reconstitute (dissolve) freeze-dried peptides. The benzyl alcohol lets a vial be used over several weeks. Essential for nearly every injectable peptide — never let a customer leave without it.

Benefits	Dissolves lyophilized peptide • Multi-week vial use (bacteriostatic) • Required for almost every injectable
Protocol	Add to the peptide vial to reconstitute; amount sets concentration (see Foundations).
Reconstitution	Not reconstituted — <i>this IS the diluent</i> . One bottle reconstitutes many peptide vials. Store at room temperature once opened; use within 28 days. (10 mL bottle = ~5 typical peptide vials at 2 mL each.)
Stacks well with	Every injectable peptide; pairs with the Peptide Supply Kit (syringes, alcohol wipes).
Talking points	“This is what you mix the peptide with. Don't forget it — and grab a supply kit for syringes and wipes too.”
Cautions	Bacteriostatic water is for reconstitution, not for IV push of large volumes. Refrigerate reconstituted vials.

Part 5 — Stacking Guide

“Stacking” means running peptides together so their effects complement each other. These are the classic, well-understood combinations. Always confirm the provider has approved any stack.

Stack name	Combination	Goal
Wolverine	BPC-157 + TB-500	Maximal injury & tissue recovery
GH Gold-Standard	CJC-1295 (no DAC) + Ipamorelin	GH optimization: muscle, fat loss, sleep
GLOW	GHK-Cu + BPC-157 + TB-500	Skin glow + healing (inside-out beauty)
KLOW	GHK-Cu + KPV + BPC-157 + TB-500	Reactive/inflamed skin + recovery
CagriSema	Cagrilintide + Semaglutide	Enhanced appetite control / weight loss
Lean-Cut	GLP drug + Tesamorelin/GH stack	Fat loss while preserving muscle
Wellness Trio	Glutathione + B12 + NAD+	Energy, antioxidant, cellular health
Focused-Calm	Semax + Selank	Daytime focus without anxiety
Longevity Base	NAD+ + Epitalon (cyclical)	Cellular energy + telomere support
Mito-Recovery	MOTS-c + SS-31 + NAD+	Mitochondrial energy & endurance

General stacking principles. Healing peptides (BPC/TB) pair with almost anything. GHRH + GH-secretagogue (CJC + Ipamorelin) is the gold-standard GH combo. Don't double up two drugs in the same class (e.g., two GLP agonists) without provider direction. Introduce one new peptide at a time so you can attribute effects and side effects.

Part 6 — Memorize & Practice (Hannah's Drill)

This section turns the manual into recall. The goal: when a customer says a goal or names a peptide, the right answer comes out automatically — no flipping pages. Drill the flashcards daily for a week, then take the self-quiz cold, then run the roleplays out loud.

A. One-line flashcards

Cover the right column. Say the answer out loud before revealing.

Peptide	Say this in one breath
Semaglutide	GLP-1 weekly shot — appetite suppression, the gentle weight-loss entry.
Tirzepatide	Dual GLP-1/GIP — stronger weight loss than semaglutide, weekly.
Retatrutide	Triple GLP-1/GIP/glucagon — most powerful weight loss, provider-watched.
Cagrilintide	Amylin analog — pairs with semaglutide (CagriSema) for extra appetite control.
AOD-9604	GH fragment — fat burning without GH side effects; gentle, non-GLP.
BPC-157	Healing peptide — tendons, gut, injuries; inject near the sore spot.
TB-500	Systemic healing partner to BPC — flexibility, whole-body recovery.
Wolverine	BPC + TB in one — complete recovery in a single shot.
GHK-Cu	Copper peptide — collagen, skin healing, hair; serum or injection. Skincare star.
GLOW / KLOW	Skin + healing blends; KLOW adds anti-inflammatory KPV for reactive skin.
Sermorelin	Gentle GHRH — boosts your own GH; nightly, better sleep first.
CJC-1295 + Ipamorelin	The gold-standard GH stack — clean GH pulse for muscle/recovery.
Tesamorelin	GH peptide that targets deep belly (visceral) fat — FDA-approved for it.
IGF-1 LR3	Advanced direct muscle-builder; provider-only, can drop blood sugar.
PT-141	Libido peptide — works in the brain, men & women, ~45 min before, as-needed.
Melanotan 2	Tanning peptide with a libido side-benefit; watch moles.
Kisspeptin	Upstream signal — tells the body to make its own sex hormones.
Semax	Focus & clarity nasal spray; raises BDNF.
Selank	Calm/anti-anxiety nasal spray; no sedation.
DSIP	Deep-sleep peptide; recovery happens during sleep.
NAD+	Cornerstone longevity molecule — cellular energy & DNA repair; inject SLOWLY.
Epitalon	Telomere peptide — short cycles a few times a year; sleep + longevity.
MOTS-c	Mitochondrial exercise-mimetic — metabolism & endurance.

Peptide	Say this in one breath
FOXO4-DRI	Senolytic — clears 'zombie' aged cells; advanced, cyclical.
Thymosin Alpha-1	Headline immune peptide — strengthens & balances immunity.
KPV	Targeted anti-inflammatory for gut & skin; non-immunosuppressive.
Glutathione	Master antioxidant — detox, immune, skin brightening.
B12	Energy shot — popular alongside weight loss.
MIC / Lipotropic	Fat-metabolism + B-vitamin support shot; weight-loss companion.
Bac water	What you mix peptides with; bacteriostatic = multi-week use. Never skip the upsell.

B. Self-quiz (answers in teal — cover them first)

Q. A customer wants to lose weight but is nervous about side effects. What's the gentle starting point and the #1 thing to pair it with?

A. *Semaglutide started low (0.25 mg/wk) with slow titration; pair with protein + resistance training to protect muscle.*

Q. What two numbers must you confirm before any dosing-in-units conversation?

A. *(1) mg of peptide in the vial and (2) mL of water added. Without both, a 'units' number is meaningless.*

Q. Which peptide specifically targets deep visceral belly fat and is FDA-approved for that purpose?

A. *Tesamorelin.*

Q. What's the gold-standard growth-hormone stack and why is Ipamorelin the preferred secretagogue?

A. *CJC-1295 + Ipamorelin; Ipamorelin gives a clean GH pulse without raising cortisol, prolactin, or hunger.*

Q. Why does PT-141 work for people that traditional ED drugs don't help?

A. *It acts centrally on the brain's arousal/desire pathways, not on blood flow — and it works for women too.*

Q. A customer asks what to mix their peptide with and how long the vial lasts. What do you recommend?

A. *Bacteriostatic water (0.9% benzyl alcohol) — it lets the vial be used over several weeks; refrigerate after mixing. Add a supply kit.*

Q. Which peptide must you NEVER make therapeutic/anticancer claims about, and what do you do instead?

A. *PNC-27 — make no claims; route every question to the provider.*

Q. Name the skincare trio a beauty-focused customer might start with.

A. *GHK-Cu (collagen/glow) + Glutathione (brightening) + Snap-8 topical (fine lines); GLOW/KLOW blends are all-in-one options.*

Q. Why do you inject NAD+ slowly?

A. *Injecting too fast causes a wave of flushing/chest tightness — harmless but very uncomfortable.*

Q. Where do our peptides come from, and what's the name you must get right?

A. *Alpha BioMed (never 'Alpha Medical'). General supplies come from McKesson.*

Q. A competitive athlete asks about BPC-157 for an injury. What must you flag?

A. *Many of these peptides (BPC, TB-500, GH secretagogues, AICAR) are on sport anti-doping banned lists — flag it proactively.*

Q. What's the difference between CJC-1295 with DAC and without DAC?

A. *With DAC = long-acting, steady GH, dosed ~weekly. Without DAC = short-acting clean pulses, daily, stacked with Ipamorelin.*

C. Customer roleplay scripts

Practice these out loud with a partner. The point isn't to memorize words — it's to internalize the **flow**: listen, then identify the goal, then educate, then route to the provider for the actual plan. Never diagnose or promise results.

Roleplay 1 — Weight-loss inquiry (nervous first-timer)

Customer: I've heard about these weight-loss shots but I'm kind of scared of them.

Staff: Totally understandable. Can I ask what your main goal is — steady fat loss, appetite control, both?

Customer: Mostly I just can't stop snacking at night.

Staff: That's exactly what GLP-1 peptides like semaglutide help with — they make you feel full sooner and longer, so the snacking urge fades. We start at the lowest dose and go up slowly, which keeps side effects mild. The provider sets your exact schedule. The one tip I always give: keep your protein up and do some resistance training, so you lose fat and not muscle.

Customer: Okay, that's reassuring. What about energy, I'm always tired when I diet?

Staff: A lot of people add a B12 shot for energy — it pairs really well. Let's get the provider to set up your plan and we'll go from there.

Roleplay 2 — Skincare cross-sell

Customer: I'm on the weight-loss program already. Do you have anything for skin? Mine's looking dull.

Staff: Yes — skincare is one of our favorites. The star is GHK-Cu, a copper peptide that builds collagen and gives skin that glow; it comes as a serum or an injection. A lot of people pair it with glutathione for brightening and a more even tone.

Customer: Is it complicated to use?

Staff: The serum is the easy on-ramp — just apply it daily. If you want the full effect we have the GLOW blend, which combines the copper peptide with two healing peptides in one. I'd let the provider confirm what fits with your current plan.

Roleplay 3 — Recovery / athlete

Customer: I tweaked my shoulder lifting and it's not healing. Someone mentioned BPC-157?

Staff: Great option — BPC-157 is our go-to healing peptide for tendon and joint stuff, and people often inject it near the area. For something stubborn we'd often pair it with TB-500, or just use the Wolverine blend that has both in one shot.

Customer: I compete in powerlifting though.

Staff: Really glad you mentioned that — several of these, including BPC-157, are on anti-doping banned lists. I'd want you to check your federation's rules before starting, and the provider can talk through options with you.

Roleplay 4 — The ‘how do I mix it’ question

Customer: I bought a peptide but I have no idea how to actually use it. It’s just powder.

Staff: No problem — that powder needs to be reconstituted, which just means mixing it with bacteriostatic water. You add the water slowly down the side of the vial, let it dissolve on its own — don’t shake it — then keep it in the fridge.

Customer: How much do I inject?

Staff: That depends on how many milligrams are in your vial and how much water you added — those two numbers set the dose on the syringe. Let’s confirm both and I’ll have the provider verify your exact dose so you’re 100% sure. Do you have syringes and alcohol wipes? Our supply kit has everything.

The three things to never skip: (1) Ask the goal before recommending. (2) Educate on what it does and how it’s taken — not a diagnosis or a cure. (3) Route every actual dosing/medical decision to the prescribing provider. When unsure, say “great question — let me get the provider to confirm that for you.”